

PROVISION OF HEALTH EDUCATION AND PROMOTION SERVICES

UNIT CODE: HE/CU/TT/CC/06/6/A

Relationship to Occupational Standards: Provide Health Education and Promotion Services

Introduction to unit of learning

This unit specifies the competencies required to Provide Health Education and Promotion. It involves identifying and assessing health education and promotion needs, developing objectives, planning, implementing, evaluating and documenting health promotion and education activities. Its main objective is to enlighten a person or a community on healthcare.

Summary of Learning Outcomes

1. Carry out needs' assessment on health education and promotion
2. Develop objectives for health education and promotion
3. Plan health education and promotion
4. Implement action plan for health education and promotion
5. Monitor and evaluate health education and promotion
6. Document health education and promotion

Learning Outcomes 1: Carry out needs' assessment for health education and promotion

Introduction to the learning outcome

This learning outcome specifies the content of competencies required to conduct dietary surveys. It comprises basics community health assessment techniques, assessing health needs of a patient, communication skills in health education and promotion, recording and Presentation assessment reports, learning activities, self-assessment questions and answers, learning activities, tools, equipment, supplies and materials and references.

Performance Standard

1. Tools for needs assessment are identified and gathered as per work place policy and WHO standards.
2. Health education and promotion need is assessed as per WHO standards
3. Needs assessment report is prepared and filed as per work place policy and WHO standards

Information Sheet

1.4.1 Basics community health assessment techniques

Community health assessments (CHAs) include a combination of quantitative demographic and health data as well as qualitative data that reflects the experiences and opinions of community stakeholders. Summarize and synthesize these data to develop a picture of overall community health and to highlight the particular health needs of various populations in your community.

Key Components

Defining demographic indicators - Indicators are the measures describing a community's health that are related to desired health outcomes; they should allow for identification of health inequities by geography, age, race/ethnicity and socioeconomic status when feasible. Before gathering data, identify meaningful indicators of health and health determinants.

Detecting disparities - Aggregate data about a population can obscure subpopulation differences. Electronic health record (EHR) data may be useful for tracking and stratifying health status by race, ethnicity and language, as well as other sociodemographic characteristics (e.g., income, disability or veteran status, sexual orientation and gender).

Reaching populations facing inequities - Some segments of a population—such as individuals who lack a stable address or who do not speak English—may not be represented in existing data. Make a specific effort to engage individuals from those populations. Consider where these individuals congregate and conduct targeted outreach in these locations.

Apply research principles to the CHA process

CHA will include a combination of primary data collected and secondary data that comes from other sources, such as the local health department. With both primary and secondary sources, there are two main types of data:

Quantitative: numerical population data that can establish a benchmark for health indicators and conditions.

Qualitative: descriptive data based on opinions and perceptions that can illustrate how people think or feel; typically gathered from interviews, focus groups, photovoice, participatory action research or town hall meetings.

Community-based participatory research (CBPR) methodology is particularly applicable to the CHA process. It is a collaborative approach to research that equitably involves all partners in the research process, recognizing the unique strengths that each partner brings and facilitating collaborative partnership through the research. CBPR principles align well with the CHA process.

Table 1: Community-based participatory research (CBPR) methodology

CBPR Principles	Relation to CHA Process
Recognizes community as a unit of identity	The unit of analysis for CHAs is the geographic community.

Facilitates collaborative partnerships in all phases of the research	CHA developers can foster long-term, collaborative partnerships with community members and stakeholders throughout the process.
Integrates knowledge and action for the mutual benefit of all partners	The hospital gains a more nuanced perspective of community health issues.
Promotes a co-learning and empowering process that attends to social inequalities	Engaging the community throughout the CHA process promotes a sense of joint ownership and equity between the hospital and community.
Involves a cyclical and iterative process	The CHA process is an ongoing cycle that should include periodic reflection and course correction.
Addresses health from positive and ecological perspectives	CHA developers are encouraged to target the social determinants of health in the community in order to address the upstream factors affecting health.
Disseminates findings and knowledge gained to all partners	CHA results are publicly available and widely distributed to participants, stakeholders and the community at large.

Develop a data strategy

When planning the data collection process, the CHA advisory committee can discuss the following considerations:

Is this the hospital's first CHA or an update from previously published CHAs?

Will the collected data be new, updated from existing data or both?

What requirements need to be fulfilled?

Is the hospital operating alone or with other partners?

Is the CHA representing one community, one hospital or multiple jurisdictions?

Will you be conducting the CHA in partnership with local health departments or other hospitals?

Is there a system in place to store and manage CHA data?

Does the hospital require additional support to compile, assess and publish the data?

What level of geographic granularity is needed to identify disparities and unmet needs?

The hospital can call on both internal and community resources to advise on data collection and provide data. This can include:

Hospital staff

Public health departments

Local colleges/universities

Research organizations

Community volunteers

Consulting firms

Public health institutes

Decide what data to include

Comprehensive CHAs include a thorough review of the statistics regarding a range of population health indicators. Data collected and analyzed should reflect the clinical and non-clinical factors that impact health. Guidelines for key metrics can be found in the County Health Rankings & Roadmaps Model:

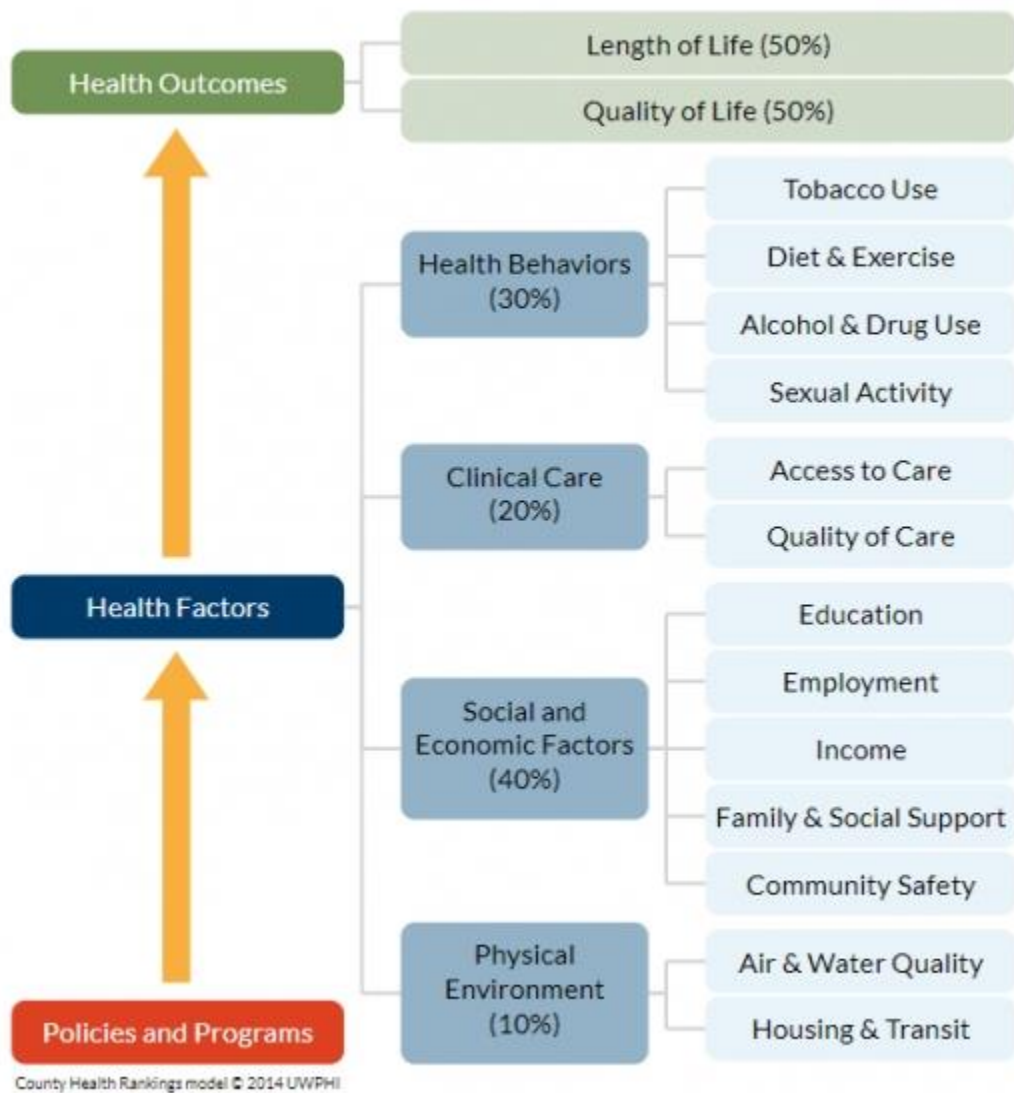


Figure 1: County Health Rankings & Roadmaps Model

Source: University of Wisconsin Population Health Institute. (2016). County Health Rankings & Roadmaps: Our approach. Accessed at <https://www.countyhealthrankings.org/explore-health-rankings/measures-data-sources/county-health-rankings-model>

Collect secondary quantitative data

Demographic and health data are compiled by local and state health departments as well as by national sources such as County Health Rankings & Roadmaps, Community Commons and the

Centre for Disease Control and Prevention (CDC). Selection of data sources will depend in part on the definition of your hospital's geographic service area. Local health department epidemiologists can assist with ensuring data quality and validity. Input from local community and neighborhood groups on access to health care, social determinants of health and other health conditions can also enhance the quality of data collected, as it is coming directly from the community.

National data sets can help you identify disparities at the local level.

Access electronic health record data

Electronic Health Record (EHR) data can provide a unique window into the health needs of community members who have received care. Accessing the aggregate EHR data will require working with the IT department to see what data they can pull from the data warehouse. Considerations include:

Benchmarking patient data against the health indicators in the hospital's service area

Aligning key indicators for the CHA with the fields being collected in the EHR

Linking patient records across the continuum of care

Tracking emergency department use

Documenting behavioral health issues and comorbidities with other chronic diseases in the EHR

Collect community-engaged primary data

Information not available in existing databases should be collected directly from your community. This is an ideal opportunity to engage your community. There are four main approaches to obtaining primary data:

Community surveys: written surveys distributed widely to the community on paper and/or online

Key stakeholder interviews: one-on-one conversations between trained facilitators and community stakeholders

Focus groups: group-based conversations of 5 to 10 participants led by a trained facilitator

Town hall meetings: community-wide meetings led by a trained facilitator

Guiding principles to consider when soliciting the opinions of community members about their community health needs include:

Involve community members in developing surveys or interview guides to ensure that questions are culturally appropriate, are understandable and will elicit desired responses. Do not use health care jargon, as most people outside of the field will not understand it.

When developing questions, make sure the questions accurately and directly address what is being measured.

Keep the wording of questions simple, with clearly defined terms. Avoid leading questions, two-part questions or questions that make assumptions about the respondent.

Test the questions on a small sample of potential respondents so they can give feedback and identify any confusing terms or suggest modifications.

Keep surveys short to reduce the time burden on the respondents and increase the response rate. Include only relevant, necessary questions.

Collect responses from a large and diverse group of individuals who are representative of the community served. If there is a significant non-English-speaking population, consider approaches that would allow those individuals to participate in their native language.

Train individuals who will be conducting interviews and focus groups to perform this work consistently and neutrally so as not to influence responses.

Develop standard processes for analyzing data. This is particularly important when coding qualitative data, as it is a fairly subjective process.

Below are some suggested practices for engaging community members in the data collection process:

Table 2: Community Surveys

Community Surveys	
Content and format	Assure respondents of confidentiality.
	Collect race, ethnicity and language data in a culturally appropriate manner.
	Ensure that survey questions are culturally appropriate and at a literacy level and language that respondents can understand.
	Review the survey draft with community members to see what needs to be modified.
	Provide versions of the survey in the languages spoken by community members.
	Consider using or modifying a validated survey instrument or questions.
	Assess regularity of health care usage as frequent users may have a unique perspective.
	Allow space for qualitative answers.
	Provide the option for respondents to be contacted for further involvement in the CHA process.
	Distribute the survey online, on paper or both. Consider using both methods if there are major segments of the community's population who do not have internet access.
Participants	Consider oversampling vulnerable populations since interventions would likely need to be focused on the needs of those groups.
	Distribute the survey where people live, work, learn and play—at churches, local businesses, health fairs, etc.
	Widely advertise the survey using social media, newspaper advertisements, etc.
	Engage community leaders to encourage participation in the survey among their constituents.
	Consider specifically surveying the patient population.

Table 3: Key Stakeholder Interviews

Key Stakeholder Interviews

Participants	Consider whom to interview:
	Interview stakeholders from a variety of sectors in the community.
	Engage clinicians—including physicians, nurses, community health workers, etc.—for interviews as they likely have insights into the health needs of patients in the hospital.
	Interview individuals representative of the community, including subgroups experiencing health disparities.
	Supplement topic areas with sparse secondary data by interviewing community stakeholders knowledgeable in that area.
	Find innovative ways to recruit for stakeholder interviews:
	Ask community leaders if they know and could provide connections to potential participants with the characteristics being targeted.
	Engage clinicians in hospitals or any associated medical groups and practices to identify patients for interviews.
	Talk to hospitals' patient and family advisory councils (PFACs).
Location	Make the location easily accessible; consider factors such as proximity to public transportation, time of day, availability of parking, child care, etc.
	Hold the interviews in a neutral space (i.e., not the hospital).
	Consider online or phone interviews to reduce barriers to participation.
Facilitator	Ensure that the interview facilitator is culturally competent and speaks the language(s) spoken by the interviewees.
	Use a facilitator who is well trained in moderating interviews, including keeping participants on topic and maintaining a neutral position.
Developing interview	Develop an interview guide so the same questions are asked across all interviews.
questions	Ask short and open-ended questions to encourage dialogue on various topics.
	Review the list of questions ahead of time with community members to ensure that questions are culturally appropriate and at a level that participants would be able to understand.
	Be aware that the interview facilitator cannot ask people to identify their health conditions.

	If possible, provide the questions to attendees ahead of time.
Conducting the interviews	Explain to participants how their input will be used.
	Establish confidentiality of the participants' responses. Especially in small communities, participants may be worried about their names being attached to their comments.
	Provide an estimated timeline of when final results will be shared.
	Ask whether the individual would like to be involved in future stages of the CHA and set the process for continued engagement.
	Establish realistic expectations for what the hospital and partners can do to address community needs.

Table 4: Focus Groups

Focus Groups	
Participants	Consider whom to sample: a cross-section of the whole community and/or more targeted groups?
	Contemplate recruiting from existing groups (e.g., PFACs, church groups).
	Find innovative ways to recruit for focus groups where people live, learn, work and play (e.g., advertisements on social media, in newspapers, on the radio, at churches, local businesses, etc.).
	Engage clinicians to identify patients for focus groups.
	Consider members of the community who may not be easily reached and brainstorm how they can be recruited.
	Encourage attendance through reminder notices.
	Limit focus groups to 10 or fewer participants to ensure that everyone's opinions can be heard.
Location	Make the location easily accessible for community members. Consider factors such as proximity to public transportation, time of day, availability of parking, child care, etc.
	Hold the focus groups in a neutral space (i.e., not the hospital).
	Consider holding virtual or phone focus groups to reduce barriers to participation.
	Focus groups should typically last no longer than 90 minutes.
Facilitator	Use a facilitator who is well trained in moderating focus groups, including keeping participants on topic, maintaining a neutral position, and making sure that everyone participates and is listened to.

	Ensure the facilitator is culturally competent and speaks the language(s) spoken by attendees.
	Consider using a facilitator from a neutral third party, so participants feel more comfortable.
Developing focus	Develop a focus group question guide, so the same questions are asked across multiple focus groups.
group questions	Ask short and open-ended questions to encourage dialogue on various topics.
	Review the list of questions ahead of time with community members to ensure that questions are culturally appropriate and at a level that participants would be able to understand.
	If possible, provide the questions to attendees ahead of time.
	Refrain from asking very sensitive questions that individuals would not want to share in a group.
Conducting the	Establish confidentiality of the participants' responses. Especially in small communities, participants can be concerned about their names being attached to their comments.
focus groups	Explain to participants how their input will be used.
	Give participants an estimated timeline of when results will be shared.
	Establish realistic expectations for what the hospital and partners can do to address community needs.
	Ask whether the individual would like to be involved in future stages of the CHA and set the process for continued engagement.

Table 5: Town Hall Meetings

Town Hall Meetings	
Participants	Advertise the meetings where people live, work, learn and play using social media, newspapers, radio, announcements and flyers, local organizations, support groups, PFACs, etc.
	If possible, offer child care for participants.
	Explain why the CHA is relevant for the whole community.
Location	Make the location easily accessible for community members. Consider factors such as proximity to public transportation, time of day, availability of parking, child care, etc.
	Hold the meetings in a neutral space (i.e., not the hospital).

	Consider coordinating the meeting with existing community or town meetings.
Finding a facilitator	Ensure that the facilitator is culturally competent, speaks the languages spoken by community members and is sensitive to attendees' needs.
	The facilitator should be well trained in moderating community meetings, including keeping participants on topic, ensuring that louder voices do not drown out others, and maintaining a neutral position.
Developing an agenda	Develop a draft agenda and questions and, if possible, distribute them to attendees ahead of time.
and questions	Ask participants open-ended questions to encourage dialogue about various topics.
	Review the list of questions ahead of time with community members to ensure that questions are culturally appropriate and at a level that participants would be able to understand.
Meeting logistics	Tell all participants how their feedback will be used and when results will be shared.
	Consider using voting devices (clickers, cell phone apps, etc.) to gain input from more community members, especially those who may not feel comfortable speaking up in a public setting.
	Establish realistic expectations for what the hospital and partners can do to address community needs.
	Provide the option for participants to be contacted for further involvement in the CHA process; this is an easy way to identify individuals who desire increased engagement.

Assessing health needs of a patient

Health Needs Assessment

A health needs assessment (HNA) is a systematic method of identifying unmet health and health care needs of a population and making changes to meet those unmet needs. Health needs assessments are used to improve health, commissioning of healthcare and other service planning, policy making and priority setting - like in a Joint Strategic Needs Assessment (JSNA) that local authorities and CCGs have to produce.

Health needs assessments lead to agreed priorities and resource allocation that will improve health, planning of services and address health inequalities. They also encourage partnership work and innovation and enable the target population to contribute.

Health needs assessments are the back bone of public health work so it is important to know how to conduct them. There are a number of ways to conduct a HNA including comparative and correlative but the most common approach is called 'epidemiologically based' where you combine epidemiology with cost effectiveness and effectiveness of interventions. It is worth noting that the aim of a HNA is to aid decision making and to promote action so it is important that the emphasis is on what actions you are recommended. As a result, many public health teams have moved away from producing a book of work that sits on a shelf to powerpoint presentations or factsheets. This helps them to influence decision makers and get public health action done. The following is my distilled version of a HNA. Remember to add an executive summary and that a lot of decision makers are really busy so keep your HNA brief and to the point with your recommendations jumping out of the page.

Sections of a Health Needs Assessment

State your **aim** - why are you doing the HNA?

Define the problem to be addressed in a defined population (and any sub-categories of the problem)

Describe the size and nature of the problem - this is your **epidemiology** section covering who's affected, risk factors, health inequalities etc.

Describe the **current services** available to address this problem

Synthesize and summarize the **evidence** from the literature on the interventions available (Literature review).

Summarize your findings from **stakeholders** - what are the viewpoints of the health professionals and others involved in providing services or interventions on this issue? (Usually this is in the form of a stakeholders workshop).

Summarize your findings from **users/patients** - what do patients want and need? (This can be from focus groups).

Summarize cost-effective solutions and **resource** implications (economic appraisal)

Recommendations for action

Identify the indicators and **outcomes** needed to evaluate change.

Further Reading: <https://www.k4health.org/sites/default/files/migra...>,
<http://www.bmj.com/specialties/health-needs-assess...>

Most doctors are used to assessing the health needs of their individual patients. Through professional training and clinical experience a systematic approach to this assessment has been developed and for use before start of treatment that is believed to be effective. Such a systematic approach has often been missing when it comes to assessing the health needs of a local or practice population.

The health needs of individual patients coming through the consulting room door may not reflect the wider health needs of the community. If people have a health problem that they believe cannot be helped by the health service, then they will not attend. For example, many people with angina or multiple sclerosis are not known to either their local general practitioner or to a hospital specialist. Other groups of patients who may need health care but do not demand it include homeless people and people with chronic mental illness.

Distinguishing between individual needs and the wider needs of the community is important in the planning and provision of local health services. If these needs are ignored then there is a danger of a top-down approach to providing health services, which relies too heavily on what a few people perceive to be the needs of the population rather than what they actually are.

Summary points

Health needs assessment is the systematic approach to ensuring that the health service uses its resources to improve the health of the population in the most efficient way

It involves epidemiological, qualitative, and comparative methods to describe health problems of a population; identify inequalities in health and access to services; and determine priorities for the most effective use of resources

Health needs are those that can benefit from health care or from wider social and environmental changes

Successful health needs assessments require a practical understanding of what is involved, the time and resources necessary to undertake assessments, and sufficient integration of the results into planning and commissioning of local services

Importance of needs assessment

The costs of health care are rising. Over the past 30 years expenditure on health care has risen much faster than the cost increases reported in other sectors of the economy, and health care is now one of the largest sectors in most developed countries. Medical advances and demographic changes will continue the upward pressure on costs.

At the same time the resources available for health care are limited. Many people have inequitable access to adequate health care, and many governments are unable to provide such care universally. In addition there is a large variation in availability and use of health care by geographical area and point of provision. Availability tends to be inversely related to the need of the population served.

Another force for change is consumerism. The expectations of members of the public have led to greater concerns about the quality of the services they receive—from access and equity to appropriateness and effectiveness.

These factors have triggered reforms of health services in both developed and developing countries. In Britain these reforms resulted in the separation of the responsibility for financing health care from its provision and in the establishment of a purchasing role for health authorities and general practitioners. Health authorities had greater opportunities to try to tailor local services to their own populations, and the 1990 National Health Service Act required health

authorities to assess health needs of their populations and to use these assessments to set priorities to improve the health of their local population. This has been reinforced by more recent work on inequalities in health, suggesting that health authorities should undertake “equity audits” to determine if healthcare resources are being used in accordance with need.

At a primary care level, through fundholding, locality commissioning, and total purchasing projects, general practitioners have become more central to strategic planning and development of health services. With this increased commissioning power has come the increased expectation from patients and politicians that decision making would reflect local and national priorities, promoting effective and equitable care on the basis of need. The Labour government has committed itself to ensuring access to treatment according to “need and need alone,” and the key functions of primary care groups will be to plan, commission, and monitor local health services to meet identified local needs.

Needs

Doctors, sociologists, philosophers, and economists all have different views of what needs are. In recognition of the scarcity of resources available to meet these needs, health needs are often differentiated as needs, demands, and supply as shown in Figure 2.

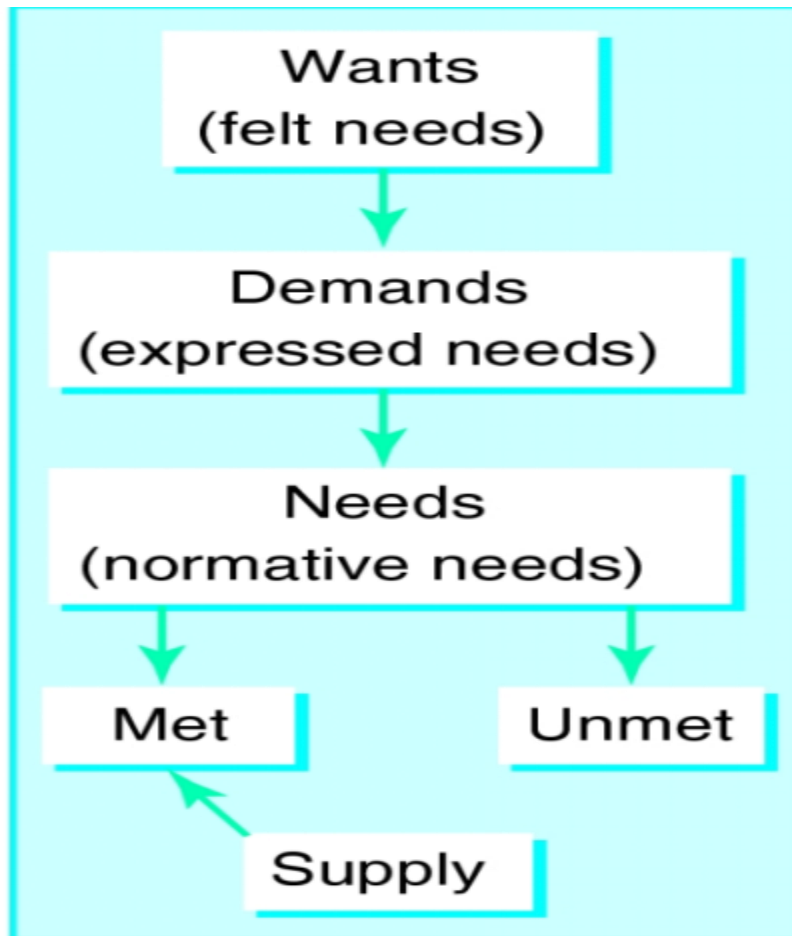


Figure 2: Health needs

Different aspects of needs

Need in health care is commonly defined as the capacity to benefit. If health needs are to be identified then an effective intervention should be available to meet these needs and improve health. There will be no benefit from an intervention that is not effective or if there are no resources available.

Demand is what patients ask for; it is the needs that most doctors encounter. General practitioners have a key role as gatekeepers in controlling this demand, and waiting lists become a surrogate marker and an influence on this demand. Demand from patients for a service can depend on the characteristics of the patient or on the media's interest in the service. Demand can also be induced by supply: geographical variation in hospital admission rates is explained

more by the supply of hospital beds than by indicators of mortality; referral rates of general practitioners owe more to the characteristics of individual doctors than to the health of their populations.

Supply is the health care provided. This will depend on the interests of health professionals, the priorities of politicians, and the amount of money available. National health technology assessment programmes have developed in recognition of the importance of assessing the supply of new services and treatments before their widespread introduction.

Need, demand, and supply overlap, and this relation is important to consider when assessing health needs as shown in Figure 3.

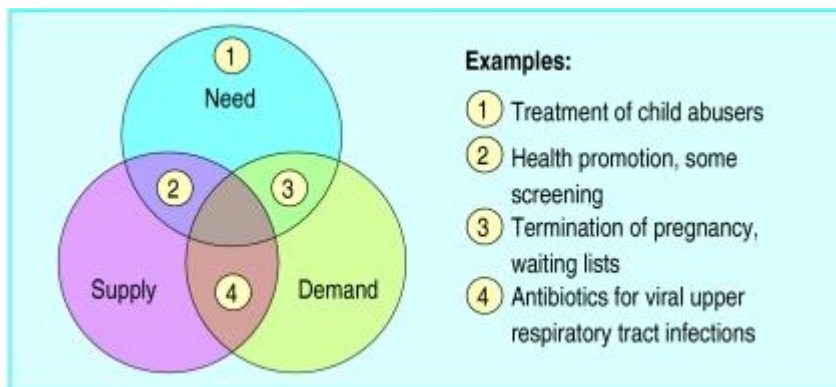


Figure 3: Relation between need, supply, and demand—central area shows ideal relation. Modified from Stevens and Raferty.

Health needs incorporate the wider social and environmental determinants of health, such as deprivation, housing, diet, education, employment. This wider definition allows us to look beyond the confines of the medical model based on health services, to the wider influences on health (box). Health needs of a population will be constantly changing, and many will not be amenable to medical intervention.

For individual practices and health professionals, health needs assessment provides the opportunity for:

Describing the patterns of disease in the local population and the differences from district, regional, or national disease patterns;

Learning more about the needs and priorities of their patients and the local population;

Highlighting the areas of unmet need and providing a clear set of objectives to work towards to meet these needs;

Deciding rationally how to use resources to improve their local population's health in the most effective and efficient way;

Influencing policy, interagency collaboration, or research and development priorities.

1.4.3 Communication skills in health education and promotion

Communication is an essential part of human life; all meaningful social interaction can be labelled 'communication'. Without communication an individual could never become a fully functioning human being. Reading, writing, listening, speaking, viewing and creating images are all acts of communication. There are also many more subtle communication activities that may be conscious or unconscious. These include expression, gesture and 'body language'. Through communication people transfer facts, ideas, emotions, knowledge, attitudes and skills to make informed decisions about their health. In this study session you will learn the basic concepts, levels, roles, principles and types of communication used in health education and health promotion.

Concepts of communication

Different writers define communication in different ways although the central concept remains the same. The word communication is derived from the Latin word 'communis' which means to form a common ground of understanding, to share information, ideas or attitudes and to impart or transmit information. The common feature of these definitions is the transmission or exchange of information. Communication implies the sharing of meaning among those who are communicating. To engage in communication is therefore to engage in the process by which

two or more people exchange ideas, facts, feelings or impressions, so that each gains a common or mutual understanding of the meaning and use of a message.

Health communication

Health communication is the art and technique of informing, influencing and motivating individuals or larger audiences about important health issues based on scientific and ethical considerations. It includes the study and use of communication strategies to inform and influence individual and community decisions that enhance health. Health communication is recognised as a necessary part of efforts to improve personal and public health. In other words, health communication encompasses the study and use of communication strategies to inform and influence individual and community knowledge, attitudes and practices (KAP) with regard to health and healthcare.

Health communication:

- initiates action
- makes needs known
- exchanges knowledge, attitudes and practices
- helps people understand issues and may change their health beliefs
- establishes and maintains relationships with health providers.

Objectives of health communication

In any type of communication, whether you are writing or speaking, trying to persuade, inform or educate, there are several general objectives. These include being understood, being accepted, and influencing an action such as a change of behaviour.

Health communication contributes to better health outcomes for individuals and for the whole community. It raises awareness of health risks and solutions, and provides the motivation and skills needed to reduce these risks. It can affect or reinforce good health practices and attitudes, giving people the information they need to make complex choices, such as selecting health

plans, care providers and treatments. Health communication also encourages social norms that benefit health and improve quality of life.

Health communication is useful in helping individuals to find support from other people in similar situations. Most importantly health communication can increase appropriate demand for and use of health services. For the community, health communication can be used to influence the public agenda, advocate for policies and programmes, and promote positive change. At the same time it can help improve the delivery of public health and healthcare services.

Health communication can be both complex and subtle and different people may focus on different areas. For example one person might generate action through helping people to understand health-related issues, and another by helping them express their needs. Both of these roles will help people to think that bringing about change is a good idea.

The role of health communication in health education and promotion

- Increases knowledge and awareness of a health issue, problem, or its solution
- Influences perceptions, beliefs, attitudes and social norms about health
- Generates effective action
- Demonstrates or illustrates health related skills
- Shows the benefit of behaviour change
- Increases appropriate use and demand for health services
- Reinforces knowledge, attitudes and behaviour
- Refutes myths and misconceptions
- Advocates for a health issue or a population group.

Levels of health communication

The more levels a communication programme can influence, the greater the likelihood of creating and sustaining the desired change. This section outlines the different levels at which health communication can take place.

Individuals

The individual is the most fundamental target for health-related change, since it is individual behaviours that affect health status. Communication can affect an individual's awareness, knowledge, attitudes, self-efficacy, and skills for behaviour change. For example, counselling on sexual risk reduction as well as HIV testing is an effective way to change behaviour and protect health.

Social networks

An individual's relationships and the groups (including family), to which they belong can have a significant impact on his or her health. Health communication programmes can work to shape the information a group receives and may attempt to change communication patterns within the group. Opinion leaders within a network are often a point of entry for health programmes such as family-based HIV counselling and testing, or peer education. Targeting health communication at social networks may result in a diffusion of innovations and network-based health strategies. It can also provide opportunities for voluntary counselling and health tests for all network members.

Organisations

Organisations include formal groups with a defined structure such as associations, clubs, worksites or schools. Organisations can disseminate health messages to their members, provide support for individuals and make policy changes that enable individual change. Examples include institution-based health programmes providing services such as voluntary counselling and health testing, or antenatal and child health services. These are often available within workplaces, schools and other institutional settings.

Communities

The collective wellbeing of communities can be fostered by creating structures and policies that support healthy lifestyles and reduce or eliminate hazards in social and physical environments.

Community-level initiatives are planned and led by organisations and institutions that can influence health such as schools, worksites, healthcare settings, community groups and government agencies.

Society

Society as a whole has many influences on individual and community behaviour, including norms and values, attitudes and opinions, and laws and policies. Society also creates a physical, economic and cultural environment.

Communicating at the society level can include using the mass media and other types of social mobilisation. For example, HIV or TB educational programmes can be achieved through school, church and workplace education programmes.

Types of communication

One-way communication

If the flow of information from the sender to the receiver is one-way the communication is dominated by the sender's knowledge and information is poured out towards the receiver. This model does not consider feedback and interaction with the sender. A familiar example of this model is the lecture method used in a classroom, where the teacher stands at the front of the class and lectures on a subject without any interaction or activities. Unless mechanisms are put in place to get feedback from the audience, many mass media communication methods are one-way.



Figure 4: One-way communication.

One-way communication isn't in itself wrong and there times when it is very useful — as Table 6 illustrates. Think of a situation when one-way communication is used effectively.

Table 6: Advantages and disadvantages of one-way communication

Advantages	Disadvantages
● Faster	● Little audience participation
● Orderly	● No feedback
● Learning of facts is authoritative	● Does not influence behaviour

Two-way communication

In this model the information flows from the sender to the receiver and back from receiver to the sender again in the other direction. Two-way communication is reciprocal, the communicant (receiver) becomes the communicator (sender) and the communicator (sender) in turn becomes a communicant (receiver). Most ordinary conversations are along the lines of this model (Figure 5). Two-way communication is usually more appropriate for problem-solving situations.



Figure 5: Two-way communication

Table 7: Advantages and disadvantages of two-way communication.

Advantages	Disadvantages
● More audience participation	● Slower, takes more time
● Learning is more democratic	
● Open to feedback	

• May influence behaviour change	
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Key terms for two-way communication

- ✓ *Sender*: the originator of each message — this could be an individual, group or organisation.
- ✓ *Message*: the idea being communicated.
- ✓ *Channel*: the means by which a message travels from sender to receiver.
- ✓ *Receiver*: the person for whom the communication is intended.
- ✓ *Effect*: the change in the receiver's knowledge, attitude or practice.
- ✓ *Feedback*: telling what they have done well or how to improve. Two-way feedback means that members of the community can tell you what you communicated well and what didn't work so well.

Health Communication

It is defined as the study and use of communication strategies to inform and influence individual and community decisions that enhance health.

Health communication includes verbal and written strategies to influence and empower individuals, populations, and communities to make healthier choices. Health communication often integrates components of multiple theories and models to promote positive changes in attitudes and behaviors. Health communication is related to social marketing, which involves the development of activities and interventions designed to positively change behaviors.

Effective health communication and social marketing strategies include the following components:

- Use of research-based strategies to shape materials and products and to select the channels that deliver them to the intended audience.
- Understanding of conventional wisdom, concepts, language, and priorities for different cultures and settings.
- Consideration of health literacy, internet access, media exposure, and cultural competency of target populations.
- Development of materials such as brochures, billboards, newspaper articles, television broadcasts, radio commercials, public service announcements, newsletters, pamphlets, videos, digital tools, case studies, group discussions, health fairs, field trips, and workbooks among others media outlets.

Using a variety of communication channels can allow health messages to shape mass media or interpersonal, small group, or community level campaigns. Health communication strategies aim to change people's knowledge, attitudes, and/or behaviors; for example:

- Increase risk perception
- Reinforce positive behaviors
- Influence social norms
- Increase availability of support and needed services
- Empower individuals to change or improve their health conditions

Examples of media strategies to convey health messages include the following components:

- Radio
- Television
- Newspaper
- Flyers
- Brochures
- Internet
- Social media tools (i.e., Twitter, Facebook, and YouTube)

Communication methods

Communication of health information is most effective when a variety of methods, approaches and materials are used. Broadly speaking, there are three main approaches:

Person-to-person contact

Captive audiences may be found at clinics, feeding centres, food-distribution centres, water-collection points and so on, where health workers and trained volunteers will be able to give advice. In nonemergency periods, health clinics, schools and workplaces may provide similar audiences. Meetings may be called for specific groups, or selected individuals may be brought together for focus group discussions on specific topics, and individual families may also be visited. The influence of existing local groups or social organizations can be very useful in increasing the impact of the information.

This direct approach, particularly if it involves some interaction between health workers and individuals, is most effective in tackling specific issues and encouraging particular changes in behaviour, and in checking that messages are seen as relevant and useful by the people concerned.

Activities suitable for person-to-person exchanges or for small groups include the discussion of personal feelings and experiences, demonstrations, story-telling, role-playing, case studies and educational games (particularly in nonemergency situations).

Teaching aids

Suitable teaching aids include printed materials, posters, films, slides, videos, murals, flannel graphs and flip charts. These are useful for transmitting information and as support to the spoken word, but must be reinforced by interaction and personal contact with members of the target audience.

Using mass communication

Radio, audiocassettes, television, video, newspapers, placards, plays, puppet shows and megaphones are effective means of communicating information quickly to a large number of people and creating awareness of a problem or idea. The relevance and impact of messages, and

the effectiveness with which they are communicated, need to be evaluated by discussion with a sample of people.

Following a disaster, mass media may be unavailable or at least severely disrupted. However, radios may be available, and in long-term emergency settlements it may be possible to produce a camp newspaper and to make arrangements with a nearby radio station to broadcast regular programmes on health issues.

Further reading: <https://www.ruralhealthinfo.org/toolkits/health-promotion/2/strategies/health-communication>

Health Education

Health education is one strategy for implementing health promotion and disease prevention programs. Health education provides learning experiences on health topics. Health education strategies are tailored for their target population. Health education presents information to target populations on particular health topics, including the health benefits/threats they face, and provides tools to build capacity and support behavior change in an appropriate setting.

Examples of health education activities include:

- Lectures
- Courses
- Seminars
- Webinars
- Workshops
- Classes

Characteristics of health education strategies include:

- Participation of the target population.
- Completion of a community needs assessment to identify community capacity, resources, priorities, and needs.
- Planned learning activities that increase participants' knowledge and skills.

- Implementation of programs with integrated, well-planned curricula and materials that take place in a setting convenient for participants.
- Presentation of information with audiovisual and computer based supports such as slides and projectors, videos, books, CDs, posters, pictures, websites, or software programs.
- Ensuring proficiency of program staff, through training, to maintain fidelity to the program model. (<https://www.ruralhealthinfo.org/toolkits/health-promotion/2/strategies/health-education>)

Health Promotion

Health promotion is the process of improving and protecting the health of the public, including individuals, populations, and communities. Health promotion can be achieved through planned activities and programs that are designed to improve population health outcomes. Health promotion programs can empower individuals to make healthier choices and reduce their risk of disease and disability. At the population level, they can eliminate health disparities, improve quality of life, and improve the availability of healthcare and related services.

Health promotion programs can be implemented in varied settings, including rural communities. Rural residents can benefit from health promotion and disease prevention programs scaled for use in rural communities, which have unique implementation considerations.

Recording and Presentation assessment reports

Contents of an assessment report

In order to be as relevant as possible, the assessment report should contain the following elements:

The aim of the assessment

The candidate's professional experience

The results of the assessment test

The explanation of the test's results that has been delivered by the candidate

The candidate's possible disagreement with this or that part of the test results or interpretation

The possible development actions in relation to the candidate's (present or future) professional occupation

Learning Activities

Practical activity

Shellemiah is a Health Extension Practitioner in the village of Kisumu. He planned to teach his community about environmental sanitation and the benefits of latrine construction. Note down which forms of health communication a Health Extension Practitioner like he might want to use to teach the community about the issue of building and using latrines.

Self-Assessment Questions

One of the services that Health Extension Practitioners are currently delivering is about HIV/AIDS and TB. Health communication is vital for delivering this service effectively. If you were conducting a health education programme on HIV/AIDS for young people, list the communication objectives you would want to achieve and justify.

Discuss some of the differences between one-way communication and two-way communication.

Give some examples of how health communication can help a. individuals, and b. communities.

Answers to Self-Assessment questions

One of the services that Health Extension Practitioners are currently delivering is about HIV/AIDS and TB. Health communication is vital for delivering this service effectively. If you were conducting a health education programme on HIV/AIDS for young people, list the communication objectives you would want to achieve and justify.

To create understanding on HIV/AIDS in the minds of young people

To help young people so that they can accept the message delivered

To get the young people to act on the required behaviour

Justification:

In any type of communication, whether you are writing or speaking, trying to persuade, inform, explain, convince or educate the general objectives would be first to create an understanding of HIV/AIDS in the minds of young people, second help them to accept the message delivered, and finally to influence the young people to change their behaviour.

Discuss some of the differences between one-way communication and two-way communication.

In one-way communication the flow of information is from the sender to the receiver. The communication is dominated by the sender's knowledge and learning is authoritative. This model does not consider feedback and interaction with the sender.

During two-way communication information flows from the sender to the receiver and back again. This model is *reciprocal* and roles are interchanged. Two-way communication is more appropriate in problem-solving situations and there is more participation and feedback, learning is democratic and it can more easily influence behaviour change. However, this type of communication takes more time.

Give some examples of how health communication can help a. individuals, and b. communities.

For individuals, health communication:

Contributes to better health outcomes

Raises awareness of health risks and solutions

Provides the motivation and skills needed to reduce these risks

Affects or reinforces good health practices and attitudes, and gives people the information they need to make complex choices, such as selecting health plans, care providers and treatments

Helps individuals to find support from other people in similar situations.

For the community, health communication:

Encourages social norms that benefit health and improve quality of life

Can increase appropriate demand for and appropriate use of health services

Can be used to influence the public agenda, advocate for policies and programmes and promote positive changes in socio-economic and physical environments

Can help improve the delivery of public health and healthcare services.

1.6 Tools, Equipment, Supplies and Materials

The following resources are provided:

Teaching charts	Sound system	Weighing scales/materials
Stationery	Recording materials/computers	Cleaning materials
Computer – internet,	VCD/DVDs -	Bronchures
Pamphlets	audio,videos	Models
Posters /flip-charts		

References

Hartwig, K., Calleson, D., and Williams, M. (2006). Unit 1, section 1.1: Definitions, rationale and key principles in CBPR. In *Developing and sustaining community-based participatory research partnerships: A skill-building curriculum*. Retrieved from <https://depts.washington.edu/ccph/cbpr/u1/u11.php>

Health Research & Educational Trust. (2016, July). Applying research principles to the community health needs assessment process. Chicago, IL: Health Research & Educational Trust. Accessed at <https://www.aha.org/ahahret-guides/2016-07-15-applying-research-principles-community-health-needs-assessment-process>

Learning Outcomes 2: Develop objectives for health education and promotion

Introduction to the learning outcome

This learning outcome specifies the content of competencies required to conduct dietary surveys. It comprises principles of health promotion, developing objectives of health promotion/education, learning activities, self-assessment questions and answers, learning activities, tools, equipment, supplies and materials and references.

Performance Standard

- I. Broad objective is drawn and documented as per WHO standards and work place policy
- II. Specific objectives are drawn and documented as per WHO standards and work place policy

Information Sheet

Principles of health promotion

The Five Principles

A Broad and Positive Health Concept:

Health should be considered broadly and not merely as the direct opposite of disease and death. It should also Embrace Dimensions of a Good Life and The Significance of Social Relations.

Underlying this principle is the fundamental assumption that humans exist within, and are influenced by, their cultural, economic and social contexts. Contemporary health problems are therefore embedded in the societal structures that surround us. This means that lifestyle and its health-related consequences cannot be treated independently of living conditions, and that a given intervention must be directed towards both lifestyle and living conditions.

Conceptualization of the “positive” stems from the World Health Organization’s (WHO) well-recognized definition of health from 1947: “Health is a state of complete physical, social and mental wellbeing, and not merely the absence of disease or infirmity.” This definition uses the concept of wellbeing to indicate the positive aspects of health. It is important to note, however, that WHO’s positive definition still includes absence of disease as one of the dimensions of health. The positive definition is, in other words, more extensive than the negative, which merely focuses on absence of diseases while still emphasizing the importance of avoiding illness.

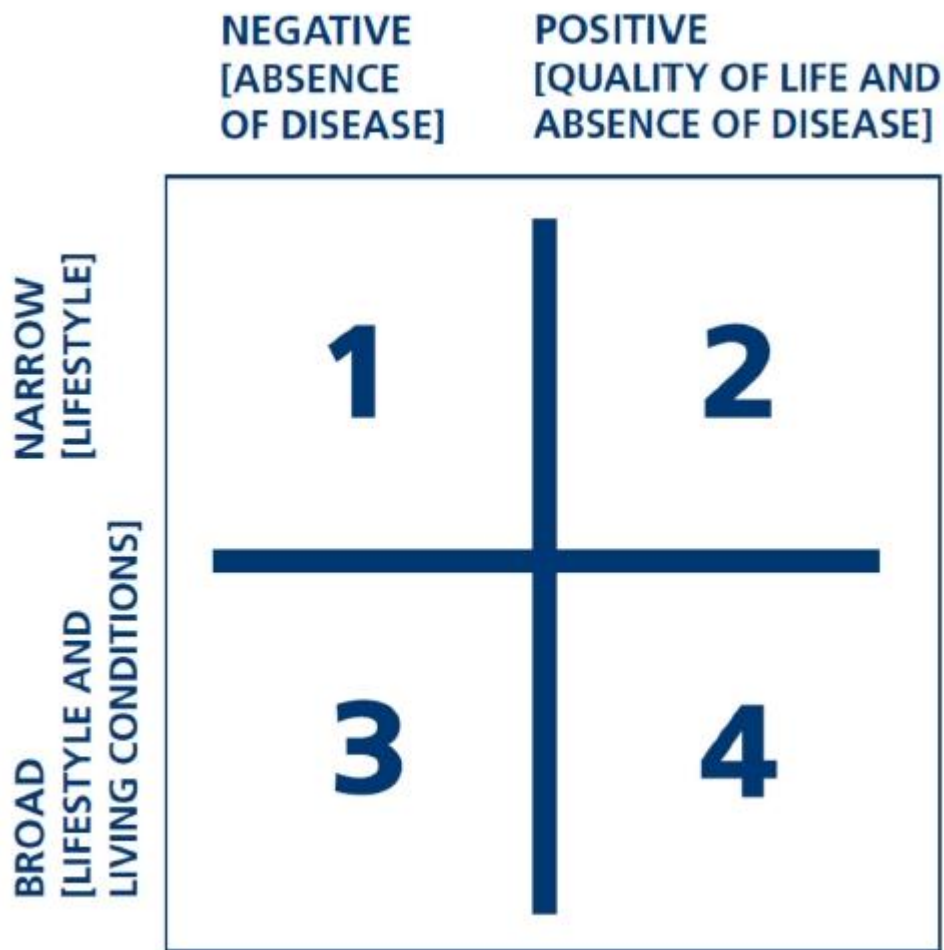


Figure 6: Four different health concepts.

Figure 6 summarizes the definition of “positive” and “broad”, with the horizontal axis representing the difference between positive and negative and the vertical axis representing broad vs. narrow. The two concepts can be combined in four different ways, and the figure therefore illustrates four different possible health concepts. The SHPR approach strives for the health concept in square 4.

For example, a “positive” definition of health is applied in relation to food and meals when both the aesthetic and nutritional aspects are addressed. Working with a “broad” concept may include the availability of healthy food in the canteen, its cost, etc., as factors influencing attempts to motivate individuals to eat healthier.

The “positive” approach to the health concept relates to the target group’s everyday language and concepts. The terms “food” and “meals” generate more interest and engagement than do “nutrition” and “diet”. Similarly, “games”, “play” and “dance” ensure greater motivation in the target groups than do “physical activity” and “exercise”. Words and concepts are not neutral. They can exclude and demotivate, or they can contribute to ownership and

internalization. Thus, a broad and positive concept of health is a determining element in attempts to engage and motivate the target group.

Participation and Involvement:

Participant involvement means ensuring the target group's influence on projects and interventions.

Participation and involvement is perhaps the most central of the health promotion principles, in the sense that sustainable health promotion change can only take place if the target group has the opportunity to develop ownership—and ownership and internalization are more likely to be achieved if the target group is actively involved in the processes from the start.

The notion of participation and involvement can imply many different things among different groups of healthcare professionals. This principle highlights the fact that development of ownership is not necessarily determined by those who take the initiative. The interesting developments may well first occur in the subsequent process, which leads to the point of decision-making. A situation in which the healthcare professional initiates a process by proposing a range of options, which are then developed and modified by the target group (e.g., patients), allows for greater involvement and strengthened ownership and empowerment. The focus for developing ownership thus shifts from “initiative” and “bottom-up” to “dialogue” and “co-creation”.

Hart notes that participation varies with individual characteristics and with context. People have different kinds of motivation, capacity and potential for participation. It is therefore important to maximise the opportunity for anyone to choose to participate at the highest level of their ability.

There are often several stages involved in health-related projects. These stages further allow for different forms of participation, and therefore a given health project often covers many different forms of participation along the way. One question could be whether participation is voluntary, and a second could relate to who takes an active lead in developing visions, ideas and proposals for solutions.

A third question could focus on who should be involved in evaluating and, if necessary, modifying the project for implementation in practice.

The principle of participation and involvement lies at the heart of diabetes prevention and management, as it places the responsibility for facilitation on the healthcare professional, whose task is to carry out an authentic dialogue with participants. During the course of this dialogue, it is both the right and the duty of healthcare professionals to contribute their professional knowledge, shape the dialogue and provide their own opinions. The dialogue must, however, facilitate discussion about mutual expectations and active participation in

the decision-making process and ensure that it is mutually respected by the participant and healthcare professional. This health promotion principle thus signals a potential alternative pathway between top-down and bottom-up: dialogue and shared decision-making.

Action and Action Competence:

Participants are given the opportunity to build skills to manage their own lives and change the conditions and structures that influence their health.

In SHPR projects the concept of action competence is an overall goal. The concept derives from an educational tradition based upon democracy, participation and a “hands-on” approach. This stands in contrast to the more behavioral tradition, where the goal is to shape individual behavior in a predetermined direction. A range of important health promotion concepts and approaches incorporate, in one way or another, a resource-oriented perspective on health issues. Attention has been directed towards, among other things, the five components described below. Collectively, these components constitute the elements necessary to achieve a high level of what we choose to call “action competence”.

Insight: A broad, positive and action-oriented understanding of health, including insight into effects, causes and change strategies within the health area.

Commitment: The desire and motivation to get involved in change processes with a view to promoting health.

Vision: The ability to think creatively and have a vision, while allowing oneself to be inspired by other scenarios.

Action experience: Experience of undertaking change processes at both individual and collective levels, including tackling and overcoming any barriers.

Critical sense: The ability to critically question information before adopting and drawing reasonable conclusions.

Action experience—i.e. direct experience with changing one’s own life or surroundings—constitutes a core element of “action competence”. A health promotion approach aimed at building up action competence should therefore allow target groups to be involved with authentic issues and challenges in order to utilize the learning potential found in real-life actions.

“Action competence” and “action” are inter-related, but should not be confused with each other.

For example, a group of children may possess the ability to act (action competence) in the context of “body and motion”, but they also require an opportunity to apply the competence in order to achieve a health-related change. In other words, there should be sufficient “room for action”, such that the developed “action competence” can unfold. Accordingly, the healthcare professional should provide space and foster the target group’s action competence. The concept of action competence is closely inter-linked with a focus on settings and enables us to look at how living conditions influence the ability to facilitate the components of action competence.

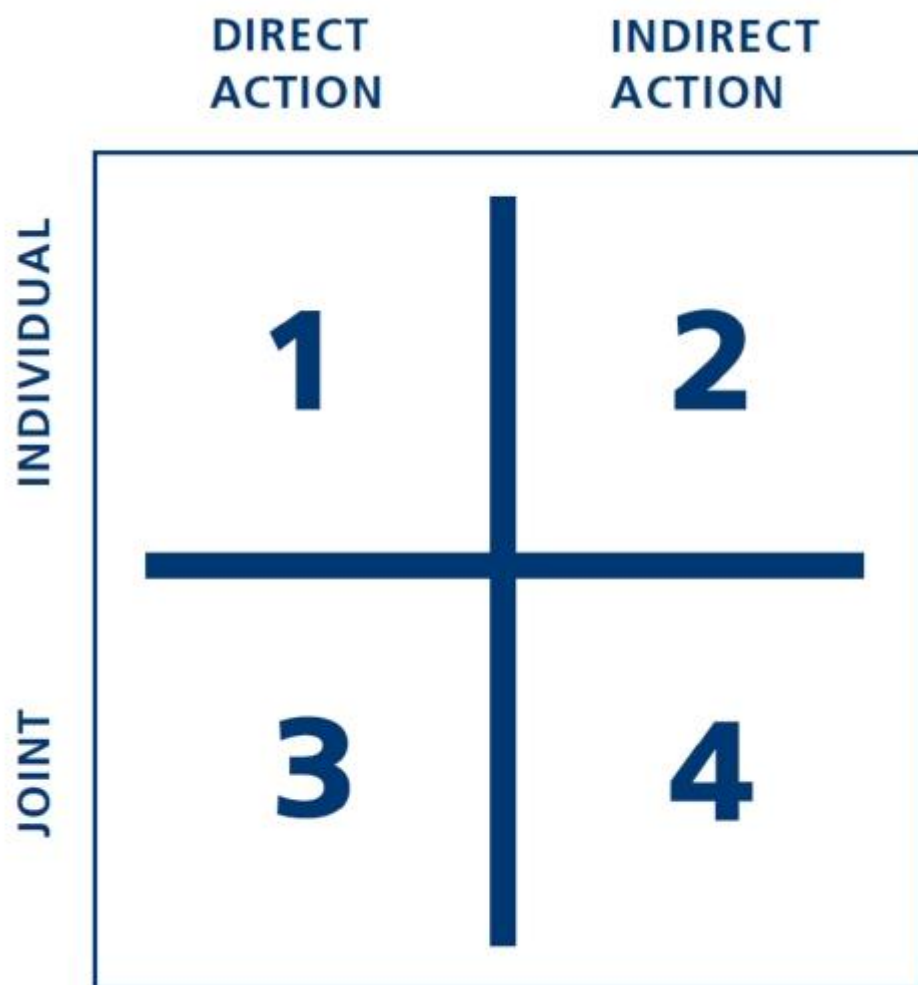


Figure 7: Four different categories of action.

Health Should be Viewed from a “Settings Perspective”:

The structural and contextual settings in which people live, work and reside influence their opportunities for healthy living

Health education cannot stand alone. Our lives and practices are influenced by the general contextual framework in which we live. The underlying aim of always applying a settings perspective is that a given intervention should encompass educational components as well as the surrounding contextual framework within which education operates. Examples of frameworks that should be included in health promotion could include environmental restructuring in different settings, e.g., the school classroom, the preschool playground, the local community centre, the physical and aesthetic environment of the clinic or the training rooms at a municipal prevention centre.

It is possible to differentiate between two different varieties of the setting concept: (1) The immediate context within which teaching or interventions occur (e.g. school, clinic, prevention centre or workplace) and (2) the context within which participants' daily lives unfold (e.g. home, local area or sport clubs).

Context, as applied in the first sense here concerns the physical, cultural and social frameworks for conducting the intervention, whereas in the second sense it relates to the framework of a daily life within which learning has to be applied. Here it is essential to focus on how physical and social structural frameworks provide optimal synergy for health promotion issues. For example, one could address how the immediate physical environment should be restructured to allow greater opportunity and motivation to encourage participants to share their experiences. For instance, does placing tables and chairs in a certain way have an effect on participation and motivation? What environmental conditions assist in creating a pleasant atmosphere that has a positive influence on motivation? How should the physical surroundings be organized to provide optimal desire to exercise among people attending a municipal prevention centre?

Equity in Health:

Not everyone has the same opportunities for leading healthy lives and changing practices. Health promotion interventions must take these differences into account

This fifth principle, which aims to increase equity in health, is strictly speaking not a methodological principle in the same sense as the four other principles. Rather, it is an ideological perspective and a core value that all projects and all of the other four principles should take into account.

There is no doubt that individuals from socially disadvantaged groups have a higher risk of developing chronic conditions such as type 2 diabetes. It is also well known that practically all methods used in diabetes management and prevention are less effective in individuals who have access to fewer resources. In other words, there is a need to direct our attention towards those who are most vulnerable and to try to understand how they can be motivated and involved as effectively as other groups.

The fifth principle therefore focuses on developing and designing tailored methods and approaches that fulfil the unmet needs of specific groups, such as residents in certain socially disadvantaged areas, or vulnerable individuals with diabetes, e.g., from ethnic minority groups or people with multi-morbidity. Moreover, this perspective also encompasses a leading global challenge that demands immediate attention in areas of the world suffering from major health-related social and financial burdens and, thus, requires effective interventions for both prevention and patient education.

For diabetes prevention and management research, the setting approach provides a view of life as it is actually lived, whether this relates to work, the family or the local community. It also sharpens the focus on the complexity associated with health promotion initiatives. Whilst other approaches to research seek to avoid or reduce the complexity of daily lives by choice of more positivist study designs or analytical methods, the setting approach welcomes complexity and seeks to actively handle it, describe it and intervene.

The underlying idea behind integrating this principle into an intervention paradigm with the four other principles is that those who have limited resources and are marginalized and vulnerable are expected to gain equally from a participant-oriented, skills-enhancing approach based on a positive, broad concept of health. However, the principles should be modified and applied in line with the actual resources available to specific target groups within their local contexts.

Developing objectives of health promotion/education

The purpose of health promotion is to positively influence the health behavior of individuals and communities as well as the living and working conditions that influence their health.

Objectives

An objective is a brief statement specifying the desired impact, or effect, of a health promotion program (i.e., how much of what should happen to whom by when).

Characteristics of good program objectives include specificity, credibility, measurability, continuity, compatibility and freedom from data constraints. The SMART acronym is an easy way to remember the key features of well-crafted program objectives; that is, good objectives are:

S pecific (clear and precise)

M easurable (amenable to evaluation)

A ppropriate (i.e., realistic)

R easonable (i.e., realistic)

Timed (specific time frame provided for achievement of objective)

Short-term Objectives

Whether an objective is short or long-term is relative to the length of time needed to achieve the program goal. As a general rule of thumb, the time frame for short-term objectives can be as short as 2–3 months up to 2 years. The time frame for the achievement of long-term objectives is usually 2–5 years.

Short-term objectives specify the short-term, or intermediate, results that need to occur to bring about sustainable long-term changes. For example, changes in knowledge need to take place to bring about longterm changes in health-related behaviours, or levels of support for a healthy public policy among decision makers need to increase before the policy can be implemented.

Here are some examples of short-term objectives:

At the end of the first year of the program, 90%of teen mothers in Natureville will know where to get birth control for free.

By the end of the first year, 80% of participating parents will have increased access to affordable, nutritious food through participation the community kitchen program and the bulk-buying club.

Long-term Objectives

Long-term objectives specify the outcomes or changes needed to achieve program goals, such as the reduction in the incidence of a health problem, or changes in health status resulting from the implementation of a healthy public policy or environmental supports.

Here are some examples of long-term program objectives:

By the end of the third year, the incidence of teen pregnancies in Natureville will decline by 50%.

To reduce the incidence of social and developmental problems associated with poor child nutrition in innercity by 2002.

Importance of developing objectives

It is essential that objectives created for your health promotion project:

are specific/non-ambiguous

are realistic.

As stated above, the objectives set out the assumptions and relationships you believe exist between what you want to happen and how you plan to make it happen.

Learning Activities

Devise learning activities

Self-Assessment Questions

List the Five Principles of health promotion

A Broad and Positive Health Concept

Participation and Involvement

Action and Action Competence

Health Should be Viewed from a “Settings Perspective”

Equity in Health

Write examples of short-term objectives:

At the end of the first year of the program, 90% of teen mothers in Natureville will know where to get birth control for free.

By the end of the first year, 80% of participating parents will have increased access to affordable, nutritious food through participation in the community kitchen program and the bulk-buying club.

What are the key features of well-crafted program objectives?

Specific (clear and precise)

M easurable (amenable to evaluation)

A ppropriate (i.e., realistic)

R easonable (i.e., realistic)

T imed (specific time frame provided for achievement of objective)

Answers to Self-Assessment questions

1). List the Five Principles of health promotion

A Broad and Positive Health Concept

Participation and Involvement

Action and Action Competence

Health Should be Viewed from a “Settings Perspective”

Equity in Health

Write examples of short-term objectives:

At the end of the first year of the program, 90% of teen mothers in Natureville will know where to get birth control for free.

By the end of the first year, 80% of participating parents will have increased access to affordable, nutritious food through participation in the community kitchen program and the bulk-buying club.

What are the key features of well-crafted program objectives?

Specific (clear and precise)

Measurable (amenable to evaluation)

Appropriate (i.e., realistic)

Reasonable (i.e., realistic)

Timed (specific time frame provided for achievement of objective)

2.6 Tools, Equipment, Supplies and Materials

The following resources are provided:

Teaching charts	Sound system	Weighing scales/materials
Stationery	Recording materials/computers	Cleaning materials
Computer – internet,		Bronchures

Pamphlets Posters /flip-charts	VCD/DVDs audio,videos	-	Models
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2.7 References

Hartwig, K., Calleson, D., and Williams, M. (2006). Unit 1, section 1.1: Definitions, rationale and key principles in CBPR. In *Developing and sustaining community-based participatory research partnerships: A skill-building curriculum*. Retrieved from <https://depts.washington.edu/ccph/cbpr/u1/u11.php>

Health Research & Educational Trust. (2016, July). Applying research principles to the community health needs assessment process. Chicago, IL: Health Research & Educational Trust. Accessed at <https://www.aha.org/ahahret-guides/2016-07-15-applying-research-principles-community-health-needs-assessment-process>